

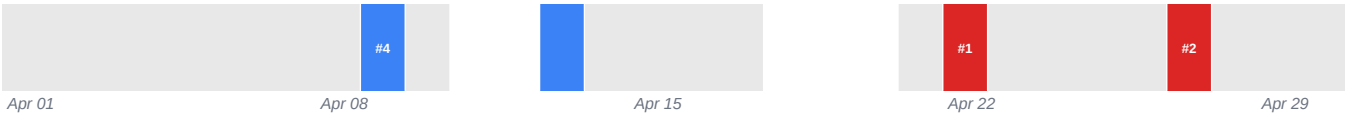
CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: Jones Period: April 1 to April 30, 2026 Report Date: May 1, 2026 Coverage: 510/720h (71%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30



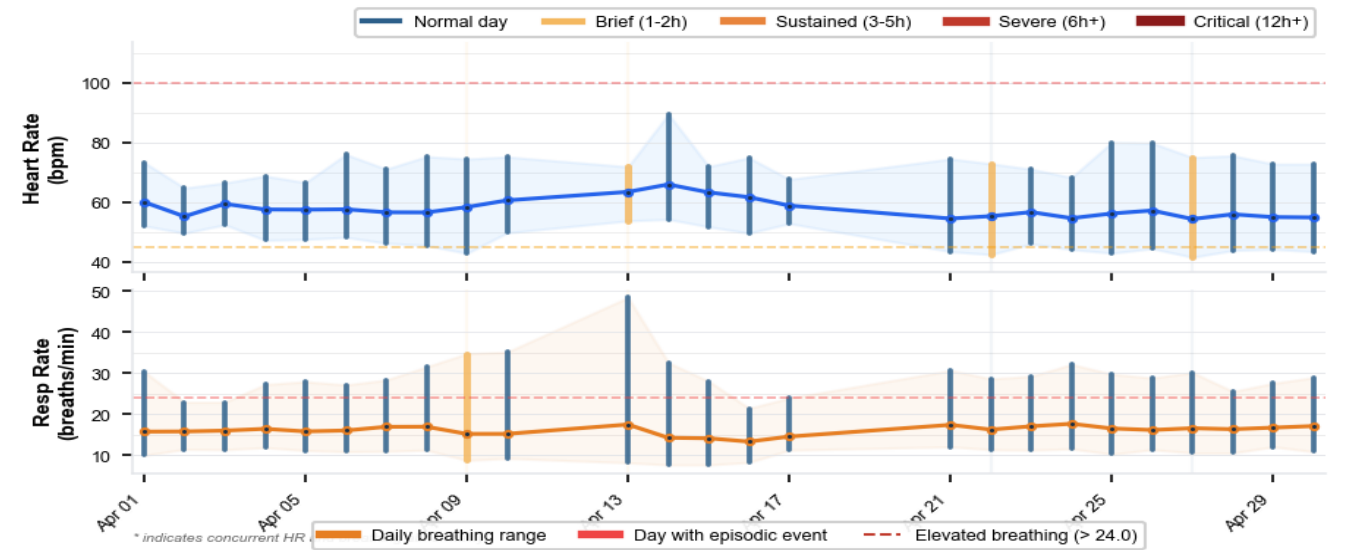
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
5	5h	Elevated Breathing	<1	71%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	42 bpm	1h	1h	1	44 bpm	Apr 22
2	Low Heart Rate	42 bpm	1h	1h	1	44 bpm	Apr 27
3	High Breathing	48 brpm	1h	1h	1	31 brpm	Apr 13
4	Elevated Breathing	35 brpm	2h	1h	2	26 brpm	Apr 09

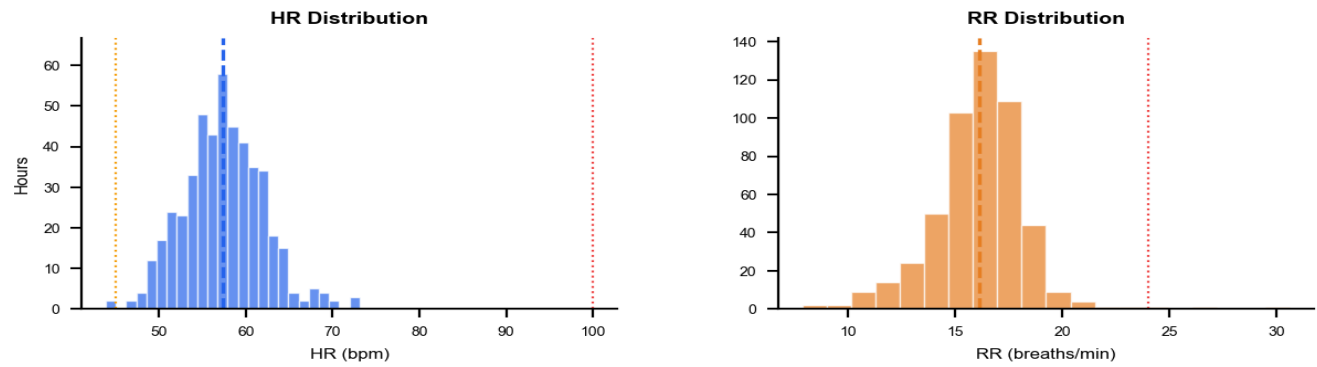
30% of readings are low confidence.

- Clinical Pattern Observations:
- 1. **Clustered pattern:** Episodic events on 4 of 30 days (13%).
 - 2. **Nocturnal pattern:** 3 of 5 eligible episodes during evening or night hours.

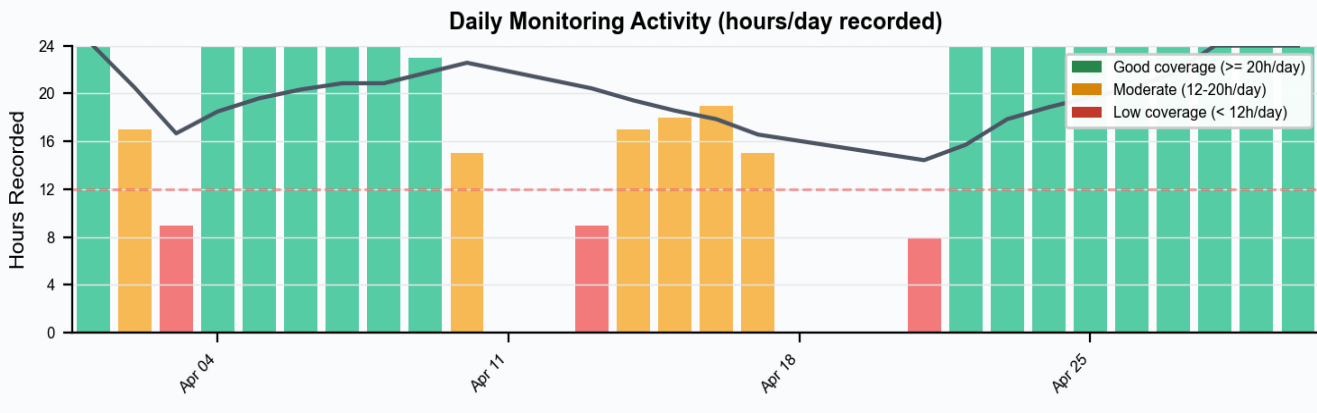


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm Low HR < 45 bpm Elevated HR > 95 bpm High HR > 100 bpm
Very High HR > 110 bpm Elevated Breathing > 24 brpm High Breathing > 30 brpm Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.